Zeporah Workflow 2: Serious Illness and Hospice-Care Preparation

Version: 1.2 — Fully integrated product, operational, and handoff specification
Primary release context: Hospice and palliative-care pilot
Reusable contexts: Hospital discharge, home health, skilled nursing, memory care, senior living, life and long-term-care insurance support
Primary user: Family Coordinator
Supporting users: Co-Coordinator, Family Helper, Family Viewer, approved Partner Support Team
Prerequisite: Workflow 1 — Family Onboarding, Consent, Roles, Permissions, and Visual Trust System

1. Workflow purpose

1.1 Family problem

When a loved one has a serious illness, begins palliative care, enters hospice, experiences functional decline, or comes home after a difficult hospitalization, family members are often expected to absorb large amounts of information quickly. They may not know which questions to ask, who is responsible for what, how to coordinate daily help, where to store instructions, how to prepare for changes, or when it is appropriate to contact the care team.

The Family Coordinator may also be afraid to ask for help, unsure how much to share with relatives, and too overwhelmed to begin a large care-planning checklist.

Zeporah turns an unclear family burden into a paced, private, family-controlled preparation process. It helps families organize what matters, prepare questions, coordinate practical help, preserve care-team instructions selected by the family, and request support from hospice or another approved organization when explicitly chosen.

1.2 Family value

By the end of this workflow, the Family Coordinator should be able to say:

- “I know who is involved and how to reach the right people.”
- “I have a place to collect questions instead of trying to remember everything.”
- “Our family can share responsibility without everyone seeing private information.”
- “I can focus on the next few actions instead of every possible future task.”
- “I can find support or request a call when I need it.”
- “Zeporah is helping us organize, not making medical decisions for us.”

1.3 Institutional value

For hospice and palliative-care organizations, Workflow 2 improves family readiness and clarity around contacts, roles, and approved resources; provides a controlled way to reduce duplicate nonclinical coordination questions; enables voluntary support requests for social work, chaplaincy, bereavement, volunteer, and caregiver support; helps long-distance relatives align when the family chooses; and extends family support without exposing private family content or replacing clinical communication.

Later configurations can adapt it for hospital discharge, home health, senior living, care transitions, and insurer care navigation.

1.4 Scope

This workflow includes entering the serious-illness, palliative-care, hospice-care, or care-transition pathway; confirming stage; selecting a manageable priority; creating care/support contacts; capturing family questions; recording family-selected care information; recording values/preferences without creating directives; identifying and assigning practical tasks; optional readiness prompts; caregiver support needs; recommendation cards; explicit-consent requests for partner support; pause, safety, and escalation paths; and permission-controlled family sharing.

1.5 Out of scope

Workflow 2 does not diagnose, interpret symptoms, recommend treatment, determine urgency, replace hospice triage/emergency services/clinical records, create or interpret legal documents, determine legal authority, transmit protected clinical information without future compliant integration and consent, promise response times absent partner commitment, or automatically notify anyone when a family updates information.

2. Participants and responsibilities

Participant	Role in Workflow 2	Default authority
Family Coordinator	Sets priorities, manages information, sharing, task assignment, and external requests	Full control within Workflow 1 permissions
Co-Coordinator	Supports planning/coordinating at delegated level	Configurable
Family Helper	Completes assigned practical tasks; sees selected information	Assigned/shared items only
Family Viewer	Receives selected updates only	No edit access
Partner Support Team	Receives voluntary approved requests; delivers services within its professional scope	No default workspace access
Zeporah System	Organizes family-selected content, suggestions, permissions, drafts, and consents	No medical, legal, or financial decision-making

3. Entry conditions and triggers

Primary entries

- Coordinator chose serious illness, palliative care, or hospice in Workflow 1.
- Coordinator selects “Begin a simple care and family plan.”
- Coordinator or Co-Coordinator opens a relevant card.
- Connected hospice/palliative partner sends optional configured invitation.
- Coordinator updates stage to serious illness, palliative care, hospice, or care transition.

Re-entry

- Incomplete plan.
- User-set task reminder.
- Helper completes task/asks a question.
- User selects “I need help” or “What should we focus on today?”
- Consented support-request status changes.

Rules

Do not assume diagnosis, prognosis, expected timeline, or end-of-life status. Stage remains editable. Opening, pausing, or not completing a task never sends a partner notice. Need-led support recommendations are nonclinical and never auto-contact a partner.

4. Workflow overview

```
Enter serious illness / palliative care / hospice pathway
        ↓
Choose one priority for today
        ↓
Set up or review care and family contacts
        ↓
Capture care-team questions and family-selected information
        ↓
Identify practical responsibilities and invite / assign help
        ↓
Record values, preferences, and what matters
        ↓
Identify caregiver and household support needs
        ↓
View optional recommendations or request a connection
        ↓
Return to a calm “Now, Next, Later” dashboard
```

This is not a required linear checklist. The family may skip, save, return, delegate, or choose one small step at any time.

5. Workflow principles

Now, next, later

- **Now:** What could make today more manageable or safer to coordinate.
- **Next:** What family may prepare in coming days.
- **Later:** Optional future items.

Never show all later-stage tasks at once or make preparation feel like a prediction of death.

Family control

Coordinator governs access. Contacts, notes, questions, and documents stay private until explicitly shared. External support requests require separate consent. A family member's request for support is not automatically visible to all other family members.

Plain-language boundary

"Your care team is the best source for medical guidance. Zeporah can help you prepare questions, keep track of information you choose to save, and find the right contact when you need support."

Emotional pacing

No nonsecurity/nonconsent task is mandatory. Every nonurgent action has Not now/Skip/Save. Recognize effort. Never use streaks, completion scores, red overdue badges, or failure language.

6. Screen-by-screen workflow

Screen 1: Pathway welcome

Trigger: Coordinator enters from dashboard, deep link, or stage selection.

Goal: Establish one-step-at-a-time care preparation.

Title: "Let's focus on what matters today"

"Serious illness can bring many decisions and responsibilities. You do not need to organize everything today. Zeporah can help you take one small step, keep important information together, and find support when you want it."

"Your care team is the best source for medical guidance. If you need urgent medical help, contact the responsible care team or local emergency services."

Actions: "Choose one next step"; "See the full care plan"; "I am not ready right now"; "I need help."

Action	Response
Choose one next step	Screen 2
See full care plan	Now/Next/Later overview, no percentage
Not ready	Save entry state, return dashboard: “You can come back whenever you are ready.”
Need help	Screen 10 support options

Screen 2: Choose one priority for today

Title: “What would make today a little easier?”

“Choose one, or choose ‘I am not sure.’ You can do the others later.”

Choices: know who to contact; keep family organized; prepare questions; practical caregiving/home help; understand preparation; emotional/spiritual/caregiver support; not sure.

Choice	Response
Contacts	Screen 3
Keep family organized	Screen 7
Questions	Screen 4
Practical help	Screen 7, then Screen 10 as relevant
Preparation	Screen 6
Emotional/spiritual support	Screen 10
Not sure	Screen 2A

Save choice as editable priority.

Screen 2A: Gentle guided choice

Title: “That is okay”

“You do not need to know the whole path right now. Most families begin with one of these small steps.”

Choices: add one contact; write one question; ask someone to help one task; find person to talk to; save for later. Save returns to dashboard and suppresses new prompt for 24 hours unless user reopens plan.

Screen 3: Care and support contacts

Goal: Family-controlled contacts, not a clinical directory.

Title: “Who is part of the support around you?”

“Add only the people and organizations your family wants to keep in one place. You can add more later.”

Categories: hospice/palliative; primary doctor/clinic; hospital/discharge; facility; pharmacy; family/friends; social worker; chaplain/clergy; employer/EAP; insurer/benefits; other.

Required: name/organization, category. Optional: contact person, phone, email, reason, hours, urgent-contact indicator only if care team instructed its use.

Actions: “Add contact”; “Add later”; “Import from my phone” when supported/consented; “Continue.”

Contacts private by default. Coordinator may share selected contacts. Partner-approved contact details may prefill only when maintained by partner and user connected; user can remove/hide them.

Screen 3A: Care-team contact guidance

Title: “Keep care-team information in one place”

“You can save the contact information your care team gives you. Zeporah does not replace their instructions. If you are unsure whether something is urgent, contact the responsible care team or local emergency services.”

Optional: reason to contact; after-hours number; main contact; written instructions to save privately. Upload warning: new documents are private by default.

Screen 4: Questions for the care team

Goal: Capture questions; do not answer or auto-send them.

Title: “Questions you want to remember”

“Write questions as they come up. Zeporah can help you keep them together for the next conversation with your care team.”

“Zeporah cannot answer medical questions. Your care team can help you understand care instructions, symptoms, medications, and what to expect.”

Input: question; optional category (comfort/daily care, medications/instructions, changes noticing, what to expect, schedule, family support, other); optional priority; visibility default private.

Actions: “Save question”; “Add another”; “I do not have a question right now”; “Request help contacting care team.”

Saved questions create dashboard cards. Group by category for review. Never auto-send. Last action routes to Screen 11.

Starter prompts on request: who to call if worried; what instructions to keep nearby; what changes to call about; how to prepare for next days; available support. Prompts are questions

for care team, not advice.

Screen 5: Family-selected care information

Goal: Private family notes, not clinical record.

Title: “Important information for your family”

“Use this space for information your family wants to remember from your care team. Please contact the care team directly if you have questions about medical instructions or if something changes.”

Cards: who to call/when; care routine/instructions; medication-related questions; upcoming visits/conversations; comfort items; other notes.

Medication boundary: “Zeporah can help you save a question or note. Do not use this space as a substitute for medication instructions from your care team.”

Inputs: note, date, optional source, access setting, optional upload/photo. Actions: save privately; choose sharing; add note; return. Default private; label notes as family-entered/unverified. Medical-advice-seeking wording routes to save as question or request contact.

Screen 6: What matters to loved one and family

Goal: Record values/preferences without creating legal/medical directives.

Title: “What matters most right now?”

“You can use this space to remember what matters to your loved one and your family. This is not a legal document or medical directive, but it can help your family prepare for conversations with the people caring for them.”

Optional prompts: comfort; desired people/updates; faith/spiritual/cultural/personal practices; visitors/routines/music/food; practical-decision values; question for care team.

Actions: save privately; share selected family; turn into care-team question; not now. Responses optional. Faith/cultural information never shared by inference. Spiritual-support selection may show resource card with save/contact/request choices.

Screen 7: Practical support and family responsibilities

Goal: Make caregiving/household work manageable while protecting privacy.

Title: “What practical help would make a difference?”

“Care can involve many small responsibilities. Choose only what would help your family right now. You can keep tasks private, do them yourself, or ask someone you trust.”

Categories: meals/groceries; transportation; child/dependent care; pet care; household; visiting schedule; supplies; phone calls/paperwork; time away from caregiving; other.

Task fields: title, category, optional description, timing, assign to, visibility, optional “I do not have anyone to ask.” Actions: create; ask someone; save later; no practical help now.

Assigned tasks notify accepted Helpers only when permitted; notification previews omit private clinical detail. Unassigned tasks stay Coordinator-only. “No one to ask” may trigger relevant support recommendations.

Delegation: select person; select existing member or invite task-only recipient; set scope; preview; send/save; recipient accepts before access.

Screen 8: Readiness and important documents

Goal: Locate information without pressure or legal claims.

Title: “Keep important information within reach”

“You do not need to gather everything now. If it would help, you can make a private list of information you may want to locate or discuss with the right professional.”

Items: contacts; insurance/benefits contact; advance-care-planning documents if any; medication list/written care instructions; family contacts; employer leave/EAP; legal/estate contact if applicable; household information.

Per item: have this; need to find; does not apply; ask someone; not now. “Have” requires no upload. “Need to find” creates private/delegated task. Zeporah does not tell family legal requirements/interpret documents. Upload is private by default with privacy reminder.

Screen 9: Caregiver check-in

Goal: Optional stated need, not surveillance/clinical assessment.

Title: “How are you doing with all of this?”

“Supporting someone can take a lot out of you. You do not have to answer this, but your choices can help Zeporah show support that may fit.”

Choices: managing okay; practical help; break/time away; alone; work/money/home responsibilities; emotional/spiritual/grief support; overwhelmed; not now.

Choice may tailor limited recommendations. Never auto-notify partner/family/emergency services. Overwhelmed opens Screen 10A.

Screen 10: Support options

Goal: Present small, transparent, need-led support set.

Title: “Support that may help right now”

“Here are a few options based on what you selected. You can save them, contact them yourself, hide them, or ask Zeporah to request a connection. Nothing is shared unless you choose to send a request.”

Categories: care team; social worker; chaplain/spiritual; volunteer/practical support; caregiver group; respite/meals/transport; EAP/leave; community/financial-hardship navigation.

Card: resource; help description; why shown; relationship label; verified cost; contact; save/hide/contact independently/request connection. Maximum four cards absent request for more. No paid placement/referral compensation/sponsorship/lead sales. Request routes Screen 11.

Screen 10A: Compassionate pause and overwhelm support

Title: "You do not have to carry this alone"

"This is a lot to manage. You do not need to finish this plan today. Choose what would feel most helpful right now."

Actions: one small step; save/return; ask family; support options; request contact from configured organization; urgent help.

Urgent message: "If someone is in immediate danger or needs urgent medical help, call local emergency services now. If you are supporting someone receiving hospice care, you can also contact the hospice's 24-hour line." Show verified line only. No triage/diagnosis/monitoring/promise of response.

Screen 11: Request support connection

Goal: Explicit, transparent, consented partner/resource handoff.

Title: "Request a connection"

"Zeporah can send a request to [recipient]. You choose what to share. Your private notes, documents, family messages, and task details are not included."

Required: request type; preferred contact method; preferred contact time. Optional: short message; voicemail/message permission.

Preview shared: selected name, contact method/time, category, optional message. Not shared: notes, documents, messages, task list, care-plan entries, unselected data.

Actions: "Send my request"; "Save draft"; "Cancel." Send requires: "I understand Zeporah will send the information listed above to [recipient]." Log recipient, fields, time, consent version, ID. Partner receives approved fields only. No real-time promise absent configured SLA.

Screen 12: Care plan dashboard

Title: "Your family care plan"

"You can take this one step at a time."

Now: one primary priority, one optional family task, one optional support action. Next: questions, practical tasks, contacts/readiness. Later: optional preferences/readiness/helper invite. Item statuses show private/shared/assigned.

Actions: add question, task, contact, find support, review access, save/return. No red overdue labels. Use “When you are ready,” “This week,” or user-set date. Passed user-set date asks “Still relevant?” Keep/new date/complete/remove.

7. Decision and branching logic

Stage	Emphasis	Avoid
Serious illness	Questions, household support, care transition, work/leave, contacts	Assuming hospice/funeral/death preparation
Palliative care	Comfort, values, caregiver support, coordination	Prognosis/end-of-life assumption
Hospice	Communication, comfort preferences, practical/caregiver/spiritual support	Symptom interpretation or triage replacement
Unsure	One step, contacts, questions, support	Forcing stage or complex plan

Solo caregiver: acknowledge, preserve privacy, offer practical support, never pressure family invitations. Multiple family: item-level sharing, updates rather than unmoderated group chat. Long-distance: remote-friendly tasks, virtual options, no default medical access. Cultural/faith/language: use voluntary preferences only; do not infer. Professional question: explain Zeporah cannot answer, offer save/contact/resource appropriate to category.

8. Permissions and sharing

Item	Default	Control
Care/support contact	Coordinator	Coordinator/authorized Co-Coordinator
Question	Creator/Coordinator	Creator within scope; Coordinator for wider sharing
Family-entered note	Creator	Creator/Coordinator under permissions
Values/preferences	Coordinator	Coordinator
Practical task	Coordinator until shared/assigned	Coordinator/authorized Co-Coordinator
Readiness item	Coordinator	Coordinator/authorized Co-Coordinator
Support-request draft/status	Coordinator	Coordinator may share summary

Sharing sequence: select person; show exact item; choose view/edit; show family-entered information reminder; confirm; audit. Partners receive no dashboard, questions, notes, preferences, tasks, documents, contacts, or updates absent separate consented request.

9. Notifications, pause, safety, partner operations, exceptions

Notifications

Only consented channels; no sensitive preview content; quiet hours; no nagging after Not now.

Event	Recipient	Message
Task assigned	Helper	“You have a new task in a Zeporah family space.”
Task completed	Coordinator	“[Name] completed a task.”
Question added/shared	Authorized person	“A question was added to your family care plan.”
Question reminder	Coordinator	“You saved a care-team question for today.”
Support request sent/acknowledged	Coordinator	“Your support request was sent/acknowledged.”
Incomplete-task check-in	Coordinator	“Would it help to revisit one small next step?”
Return after pause	Coordinator	“When you are ready, your family plan is here.”

Pause and safety

Save drafts; never interrupt a long pause aggressively. On return: “Your progress is saved. You can continue, choose a smaller step, or leave this for later.” Three incomplete opens in 14 days may show optional smaller-step/support card; no partner contact.

Overwhelm opens support path. Immediate danger shows emergency/hospice-line instruction and product limitation. No clinical monitoring, diagnosis, triage, or risk scoring.

Partner operations

Partner preconfigures program, verified contacts, accepted request types, queues/roles, hours/SLA, escalation, resources, external-resource governance, privacy/data agreement, language/access paths.

Request routing: care team → care queue; social worker → social work; chaplain → spiritual queue; practical help → social work/volunteer/resource; caregiver/bereavement → appropriate queue; contact navigation → family support/customer service.

Partner receives only ID, category, Coordinator name, preferred method/time, optional message, submission time, program ID. Never receives notes, questions, docs, tasks, preferences, helper names, discussions, inferred emotional state, or unapproved data. Status updates limited to received, acknowledged, contact attempted, connected, referred, closed.

Exceptions

Contradictory care notes: family-entered/unverified, retain history, direct to care team. Team/partner changes: archive/update contacts, never auto-transfer data. Unknown team: offer consented contact-help if connected. Multiple questions: author/visibility preserved; no autosend. Request access: Coordinator decides. Sensitive data in free text: warn against

account/SSN/password data. No family help: support resources, no repeated invitation pressure. Out-of-area: disclose limits/offer configured alternatives. Change to recent loss: confirmation, compassion, preserve/archival choice, no notices.

10. Data, analytics, and acceptance

Data minimization

Collect only optional contacts, questions, notes, preferences, tasks, readiness status, support needs/preferences, and request contact details when family elects to add them. Do not require diagnosis, prognosis, medical record, medication regimen, SSN, legal authority, insurance number, bank data, or financial statements.

Ask only what a selected feature needs. Keep new notes/docs private. Send only consent-preview fields. De-identify analytics where feasible; provide edit/export/deletion consistent with policy/law/contracts.

Metrics

Family: entry, first action, time to contact/question/task, captured questions, created/completed tasks, helper assignment, resource relevance, support-request use/completion, preparedness/confidence, Not-now return. Organization: partner activation, voluntary requests, response time, resource use, repeat nonclinical contacts where allowed, satisfaction, staff navigation time, referral completion. Safety/privacy: intended sharing, permission reversals, consent-preview views, emergency-boundary displays, access attempts, audit completeness.

Audit events: care_pathway_entered/saved; priority selected; contact added/shared; question added/shared; note created/shared; values saved; task created/assigned/completed; readiness updated; check-in completed; recommendation shown/saved/hidden; connection started/share preview/consent/sent/withdrawn; partner status; overwhelm opened; emergency shown; stage changed.

Acceptance

Ready when Coordinator can start with one priority; boundary is clear; contacts/questions/notes/preferences/tasks/readiness are paced; all nonurgent steps save/skip; privacy defaults hold; item sharing is granular; nothing auto-sends to partner; recommendations rely on stated need/intentional action; consent previews exact recipient/data and audit; partner contacts maintained; overwhelm saves/does not monitor; stage changes preserve/no notify; partner queue is limited; mobile/accessibility/plain-language design works; metrics instrumented.

11. Workflow integration and handoffs

11.1 Integration purpose

Workflow 2 is a connected stage of one Zeporah journey, not a standalone care-planning tool. It uses the private space, roles, permissions, preferences, and consent rules from Workflow 1. It creates care-preparation and coordination items that later workflows use only within active access and sharing boundaries.

Families do not start over when moving between care preparation, family coordination, support, recent loss, or later life administration. The person, people, selected priorities, and choices remain available under family control.

11.2 What enters

Entering object/context	Source	Use in Workflow 2
Workspace ID	Workflow 1	Associates care items with private space
Authenticated user/current role	Workflow 1	Determines view/create/edit/share/assign rights
Membership	Workflow 1, updated by Workflow 3	Enforces current scope only
Privacy defaults	Workflow 1	Applies private-by-default to new content
Journey state	Workflow 1/user update	Drives stage content, no prognosis inference
Optional name/relationship	Workflow 1	Family-facing personalization only
Language/accessibility/reminders	Workflow 1	Presents accessible UI and notifications
Optional location	Workflow 1	Resource matching only, no external disclosure
Priorities/support needs	Workflow 1/earlier session	Prioritizes optional actions, never auto-contacts
Recommendation preferences	Workflow 1	Controls nonessential cards
Partner connection/request pathways	Workflow 1	Enables voluntary requests, no workspace access
Prior consent/audit history	System layer	Enforces rules, not content for partners
Existing objects	Prior W2/shared workflows	Displays only items current user can access

11.3 What it creates or updates

Object	Outcome	Default visibility	Controller
Care-plan priority	Today’s focus	Coordinator	Coordinator/authorized Co-Coordinator
Contacts	Care/support directory	Coordinator	Coordinator/authorized roles
Care-team questions	Family question list	Creator/Coordinator	Creator within scope; Coordinator sharing

Object	Outcome	Default visibility	Controller
Family care notes	Family-entered unverified information	Creator	Creator/Coordinator per permissions
Values/preferences	Voluntary comfort/cultural/faith/practical wishes	Coordinator	Coordinator
Practical tasks	Ownership, timing, status, visibility	Coordinator until assigned/shared	Coordinator/authorized Co-Coordinator
Readiness items	Status locating relevant information	Coordinator	Coordinator/authorized Co-Coordinator
Caregiver check-in	Voluntary support need	Coordinator/individual per policy	User/Coordinator per design
Resource engagement	Save/hide/contact/request choices	Private according to context	User/Coordinator external control
Support-request draft	Recipient/category/contact preference/message	Coordinator	Coordinator
Consent record	Exact recipient/fields/time/version/ID	Audit record	System
Journey stage	Voluntary stage update	Family-facing only	Coordinator

11.4 What it sends forward

Available downstream	Later workflow(s)	Conditions
Current priorities and Now/Next/Later state	W3, W4	Supports coordination/return; not partner-visible
Contacts	W3–W6	Retain item permissions
Questions	W3, W4	Organize/prepare request only; no auto-send
Family notes	W3, W5, W6	Private/unverified; only permitted roles
Values/preferences	W3, W5, W6	Family-selected planning/resource use; no automatic external sharing
Practical tasks/status	W3, W5, W6	Shared system; permissions persist
Readiness status	W3, W5, W6	Optional future surfacing only
Caregiver needs/preferences	W4 + recommendations W3–W6	Tailoring only; no auto-referral
Saved/hidden resources	Recommendation service W3–W6	Prevent repetitive/irrelevant suggestions; private
Request summary/status	W3–W6	Family sees its own status; partner sees only that request
Updated journey state	W3–W6	Changes pathways; no automatic notice
Specific audit/consent history	System layer	Does not authorize new disclosure

11.5 What remains private or inactive

Contacts, questions, notes, tasks, readiness, preferences, and check-ins remain private until shared/assigned. Family notes are not clinical records. Questions are not auto-delivered. Values/faith/culture preferences are not inferred/shared. Overwhelm/isolation/financial-stress reports do not notify anyone. Recommendation browsing/actions do not create leads/referrals/notifications. Partner gains no continuing workspace access after a submitted request. Every new external handoff needs new field- and recipient-specific consent. Stage changes do not notify anyone.

11.6 What triggers another workflow

Trigger	Resulting workflow/capability	Continuation behavior
Create/share/assign practical task	Workflow 3	Preserve item visibility/owner; family-friendly delegation choice
Invite/manage helper for delegation	Workflow 3 + W1 permission service	Access preview; no unapproved access
Create/update family communication	Workflow 3	Controlled update, not automatic group chat
Need help/overwhelmed/no helper/repeated incomplete task	Workflow 4	Save draft; smaller step/delegation/support; no auto-escalation
Request connection/contact help	Consent-handoff service governed by W4	Exact recipient/data preview and consent
Stage changes to recent loss	Workflow 5	Confirmation, condolence, preserve/archival choice, no notices
Stage changes to life after loss	Workflow 6	Pacing, retain relevant objects under permissions
Access changed	W1 governance service	Current permissions apply prospectively
Resource save/hide	Shared recommendation service	Update private recommendation history

11.7 What the family sees

Families never see “Workflow 2,” “handoff,” “module,” or internal technical labels. They see one care-and-family experience.

Examples:

- “You have one important contact in one place. Would you like to add a question or return to what matters today?”
- “Would you like to keep this private, do it yourself, or ask someone you trust to help?”
- “Your question is saved for the next time you speak with the care team.”
- “This is a lot. Your progress is saved. We can make this smaller, find support, or leave it for later.”

- “We are so sorry for your loss. You can move at your own pace. We can focus on what matters right now.”

Dashboard combines: What matters today; People and tasks; Questions and information; Find support; Later. Returning from support returns to exact preserved task/draft when possible.

11.8 What a partner sees

Partner sees only one family-initiated, consented request: ID, request category, Coordinator name, preferred contact method/time, optional message, submission time, program ID. Partner does not see dashboard/stage, contacts, questions, notes, docs, tasks, readiness, values/preferences, caregiver check-ins, recommendation history, other family identities/roles, notification activity, audit history, or future content. It can return only limited status: received, acknowledged, contact attempted, connected, referred, closed. No staff notes, clinical data, or internal details return through this integration.

12. Handoff to other workflows

Workflow 3: Family coordination, communication, tasks, and documents

Trigger: Coordinator creates/assigns task, shares contact/question, invites helper, or selects keeping family organized.

Available under current access: Tasks/statuses; explicitly shared/needed contacts; explicitly shared questions; selected updates/preferences; current roles/permissions.

Not automatically shared: Private notes/docs, full care plan, check-ins, values/preferences, partner request data.

Family-facing continuation: “Would you like to keep this private, do it yourself, or ask someone you trust to help?”

Workflow 4: Caregiver overwhelm, compassionate pause, and human-support escalation

Trigger: User selects need help/overwhelmed; no helper; or configured nonintrusive repeat-incomplete-task pattern.

Available: Private current draft/task state; user-selected support category; recommendation preferences; partner connection/request options.

Not automatically shared: Emotional state, draft content, tasks, contacts, notes, or preferences.

Family-facing continuation: “This is a lot. Your progress is saved. We can make this smaller, ask someone you trust, find support, or come back later.”

Workflow 5: Immediate post-death guidance and funeral-period coordination

Trigger: Coordinator intentionally changes stage to recent loss.

Retained: Roles/permissions; contacts; notes/preferences; tasks/readiness; support preferences/partner connection.

Choices: Keep plan active, archive for now, or continue to immediate next steps.

No automatic notices to helpers, hospice, hospital, insurer, or resource.

Family-facing continuation: “We are so sorry for your loss. You do not have to face every responsibility at once. We can begin with what matters right now.”

Workflow 6: First-year bereavement, estate, benefits, and life-administration navigation

Trigger: Coordinator selects life after loss or later-stage path.

Available: Existing membership/permission rules; relevant contacts/tasks/documents/preferences/support history under current scopes. Historic consent is never reused for a new external handoff.

Family-facing continuation: “Choose one next step for today. You can return to anything else later.”